

ISCHAEMIC HEART DISEASE: ROUTINE CARE

Assess the patient with ischaemic heart disease

Assess	When to assess	Note
Symptoms	Every visit	- If recent episodes of chest pain/discomfort, assess ischaemic heart disease symptoms if not already done ↗ 137. - Ask about leg pain ↗ 65 and symptoms of stroke/TIA ↗ 136.
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↗ 143.
BP	Every visit	If known hypertension ↗ 133. If not, check BP: if $\geq 140/90$ ↗ 132.
Diabetes risk	At diagnosis and yearly	If known diabetes ↗ 130. If not known with diabetes, check glucose ↗ 17.

Advise the patient with ischaemic heart disease

- If patient has had a heart attack, s/he can resume normal daily and sexual activity 1 month after heart attack if symptom free.
- Emphasize the importance of lifelong adherence to medication. If difficulty with adherence, give adherence support ↗ 173.
- Ensure patient knows how and when to use sublingual nitrates. Explain that they are not addictive and can also be used before activities which may provoke chest pain.
- Patient should avoid non-steroidal anti-inflammatories (like ibuprofen), as they may precipitate chest pain.
- If < 55 years (man) or < 65 years (woman), advise the first degree relatives (parents, siblings, children) to have CVD risk assessment.

Treat the patient with ischaemic heart disease

- Give **influenza vaccine** 0.5mL IM yearly. Check that patient is up to date with COVID-19 vaccine.
- Help the patient to manage his/her CVD risk ↗ 129.
- Give **aspirin** 150mg daily for life. Avoid if peptic ulcer, dyspepsia, kidney or liver disease.
- Give **simvastatin**¹ 40mg at night. If on amlodipine, give instead **simvastatin**¹ 20mg at night (no dose adjustment needed for rosuvastatin, pravastatin, atorvastatin). Avoid if pregnant or liver disease.
- Give **atenolol** 50mg daily, even if no chest pain/discomfort. Avoid in asthma, COPD, heart failure, peripheral vascular disease.
- If unstable angina or following heart attack: if signs of heart failure, give **enalapril** 2.5mg 12 hourly and increase slowly to 10mg 12 hourly. Avoid if pregnant, angioedema or renal artery stenosis.
- If patient has **stable angina**, treat using stepwise approach as in table below:
 - If chest pain/discomfort controlled, continue same medication and dose.
 - If still gets episodes of chest pain/discomfort, increase to maximum dose. If symptoms continue after this, add next step. Ensure patient is adherent before increasing medication.

Step	Medication	Dose	Maximum dose	Note
1	Isosorbide dinitrate with chest pain and before exertion	5mg sublingual with angina	3 doses of 5mg with each episode of chest pain	If chest pain on exertion, rest and take 1st dose. If chest pain persists, take a further 2 doses 5 minutes apart. If no better 5 minutes after 3rd dose, patient must seek medical attention urgently.
	Atenolol	50mg daily	100mg daily	Titrate to resting pulse rate of 60 beats/minute. Avoid if asthma, COPD, uncontrolled heart failure, peripheral vascular disease or if side effects (headache, cold hands/feet, impotence, tight chest, fatigue) are intolerable. Use amlodipine instead.
2	Add amlodipine	5mg in the morning	10mg daily	Avoid if heart failure, discuss with specialist. Reduce simvastatin dose to 20mg at night.
3	Add: isosorbide mononitrate	10mg at 8am and 2pm	20mg at 8am and 2pm	-
	or isosorbide dinitrate	20mg at 8am and 2pm	30mg at 8am and 2pm	-

- If atenolol and amlodipine contra-indicated/not tolerated or chest pain/discomfort persists on full treatment, refer to specialist.
- Review monthly until symptoms controlled. Then review 3-6 monthly.

¹If on lopinavir/ritonavir or atazanavir/ritonavir, avoid simvastatin, give instead **atorvastatin** 10mg at night.